

Rural Health Transformation Program Updates

September 2026



Stevens Amendment

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General Information

- Slides from presentation will be added to RHTTP Website
- Questions can be entered into the Chat
- Website being updated regularly – check for updates
 - Surveys (Important that we hear from you!)
 - Important Dates Section
- List Serve still in development

CMS Reporting Timelines

- Full Report to be shared pending CMS approval
- Budget Year 1 Annual Report: Submitted 8/31/2026
 - Checkpoint by initiative: Summary, metrics, number served, evidence
 - State Policy Action commitments: Evidence
 - Use of Funds reporting: obligated / spent by funding bucket and type of entity
 - Success, Sustainability, and Rural Definition
- NCC Year 2 Continuation Request: Submitted 8/31/2026
 - Updated grant forms
 - Project narrative and budget narrative updates for Year 2 funding request
- Quarterly Report: Due 11/29/2026 – Include all obligated funds up to 10/30

Vendor Application

- All selected for grant funding will need to be a registered vendor to receive payment.
- You will need to be registered when signing the Agreement.
- You can find the new vendor registration at this link:
<https://www.doa.la.gov/media/qswdhkoj/lagov-new-vendor-regisration.docx>
- If you are unsure if you are a vendor and/or need to maintain your account, you can find information at this link or contact the Office of State Uniform Payroll Vendor Section directly at DOA-OSRAP-LAGOV@la.gov or 225-342-1097 : [Vendor Information - Louisiana Division of Administration](#)

Rural Clinician Credit Bank

Awards

Year 1 NOFO: \$10.0M advertised, \$12.70M awarded



RURAL CLINICIAN CREDIT BANK · Year 1 NOFO overview · As of 8/28/26

136

applications received

70

eligible after screening

53

awards made, 76% of eligible

\$12.70M

total awarded

The solicitation

Item	Year 1
Allocation of record	\$10.00M
Application close	6/25/26
Applications received	136
Eligible after screening	70
Amount requested by eligible applicants	\$17,848,171
Awards made	53
Amount awarded	\$12,701,996
Ceiling applied per multi-entity system	\$1,500,000
CEAs due for signature	9/15/26
Year 2 application opens	Nov 2026

Per-clinician caps

Role	Cap per clinician
Physician	\$75,000
Dentist	\$60,000
Nurse practitioner	\$50,000
Mental and behavioral health	\$45,000
LPN	\$20,000

Employer match ramp

	Y1	Y2	Y3	Y4	Y5
RHTP share	75%	75%	50%	50%	25%
Employer share	25%	25%	50%	50%	75%

How awards were set

ldh.la.gov/page/rural-health-transformation-program

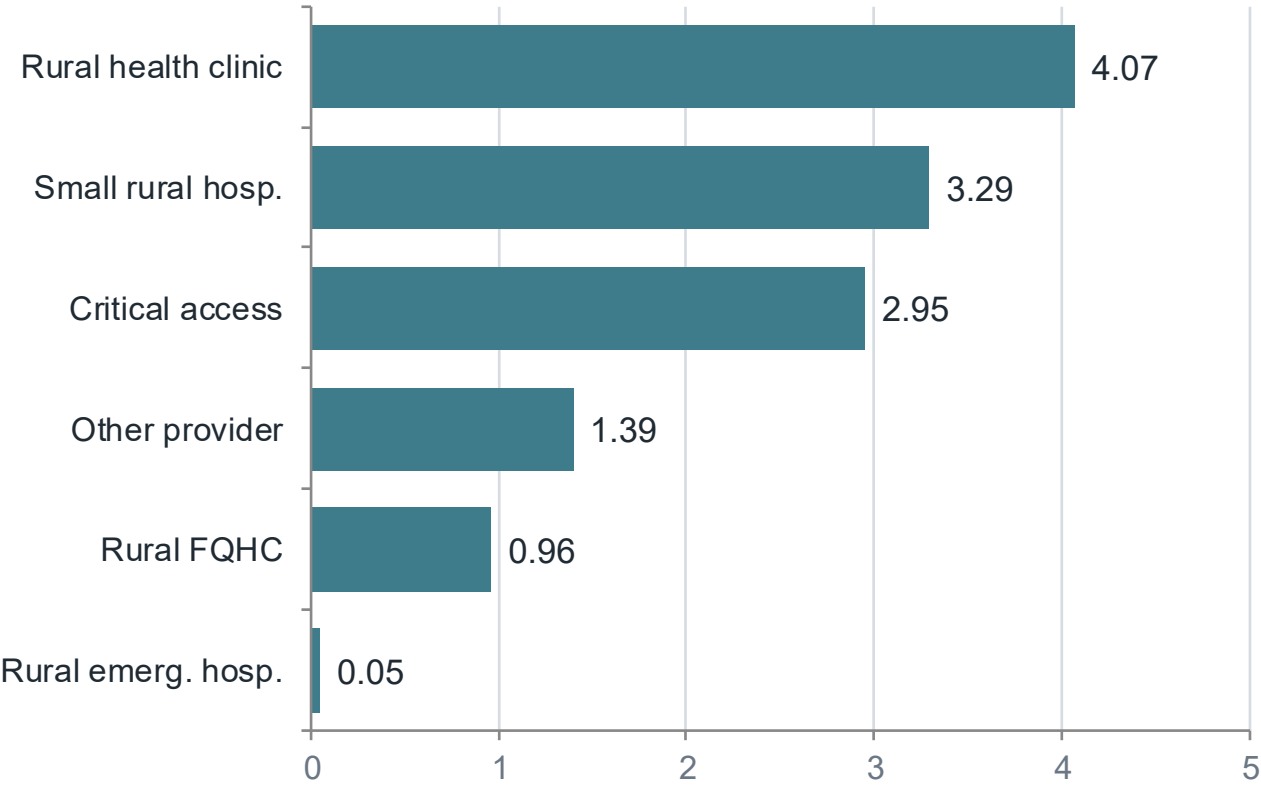
1. Two program managers screened every application for eligibility. Incomplete and ineligible applications were not scored. 2. Reviewers scored independently. Raw scores were averaged across reviewers, then weighted once into a 0-100 score plus an optional bonus. 3. The highest weighted score, 92.3, was set at 100% of its request. Every other award was reduced to that applicant's score as a percentage of its own request. 4. A \$1.50M ceiling was applied to multi-entity system. 5. Applicants received the scoring logic, denied applicants may reapply in Year 2, targeting November open timeframe.

Breakdown by facility type

RURAL CLINICIAN CREDIT BANK · Funded facilities by type · 53 awards, \$12,701,996

Facility type	Awards	% of awards	Amount awarded	% of dollars	Average award
Rural health clinic	15	28%	\$4,066,202	32.0%	\$271,080
Small rural hospital	8	15%	\$3,291,553	25.9%	\$411,444
Critical access hospital	11	21%	\$2,948,962	23.2%	\$268,087
Other provider / support	15	28%	\$1,392,154	11.0%	\$92,810
Rural FQHC	3	6%	\$958,125	7.5%	\$319,375
Rural emergency hospital	1	2%	\$45,000	0.4%	\$45,000
Total	53	100%	\$12,701,996	100%	\$239,660

Dollars awarded by type (\$M)



Ambulatory vs. Hospital

Hospital settings

20 awards · \$6,285,515 · 49% of dollars

Clinic and outpatient settings

33 awards · \$6,416,481 · 51% of dollars

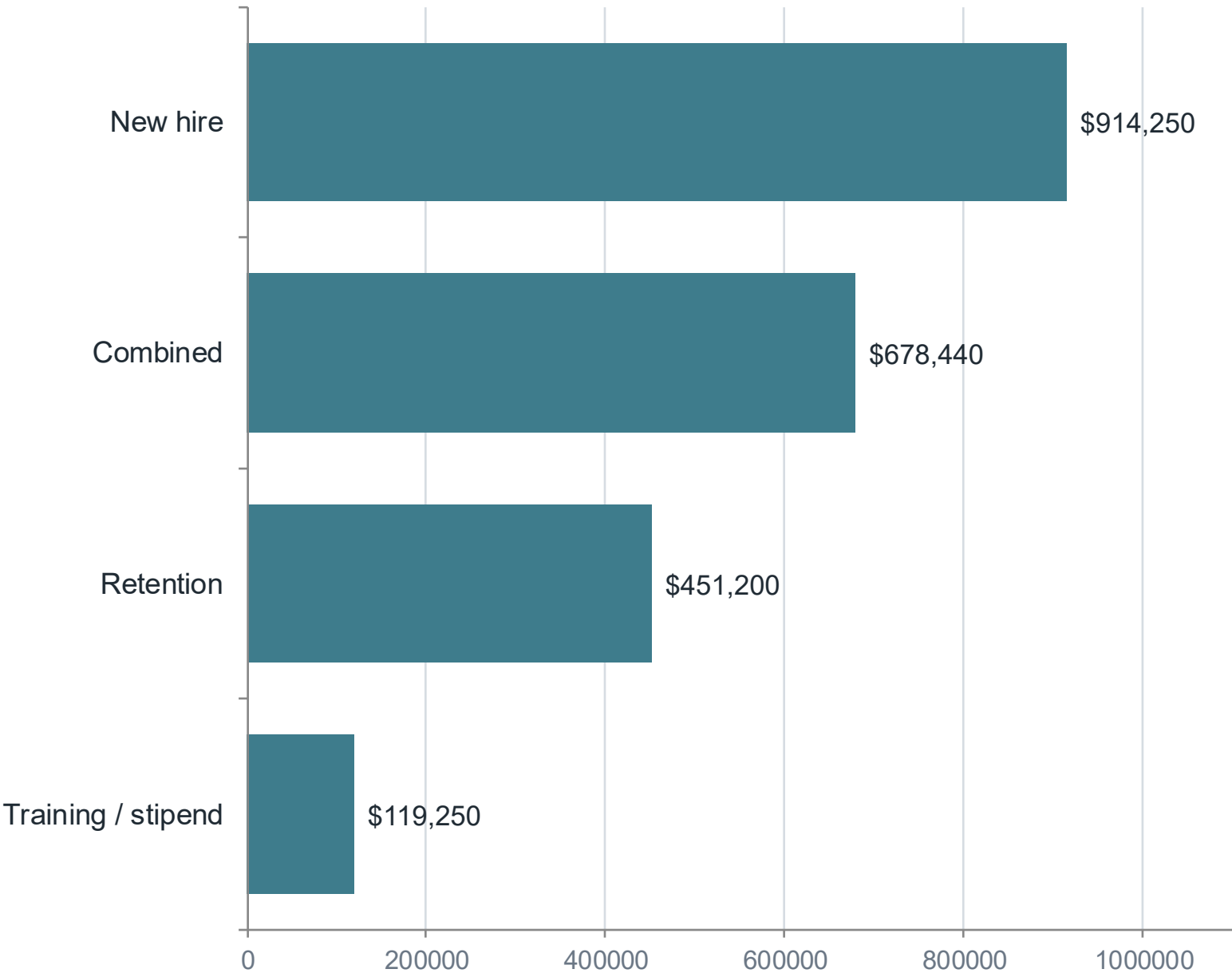
Awards split evenly between hospital and clinic settings, but the money per site does not: small rural hospitals average \$411K against \$93K for the other provider and support category, which is dental, behavioral health, therapy and EMS.

Top categories include physicians and nurses

RURAL CLINICIAN CREDIT BANK · Funded positions by clinician type

Clinician type	Year 1 RHTP	% of parsed
Physicians	\$619,500	28.6%
LPN	\$476,440	22.0%
Registered Nurses	\$340,500	15.7%
NP / APP	\$178,850	8.3%
Dentists	\$160,000	7.4%
Allied health / therapy	\$118,750	5.5%
Paramedic / EMT	\$108,000	5.0%
Mental and behavioral health	\$84,600	3.9%
Not classifiable from the line text	\$45,000	2.1%
Administrative / non-clinical	\$22,500	1.0%
Dental hygienists	\$7,500	0.3%
CNA / nurse aide	\$1,500	0.1%
Total parsed	\$2,163,140	100%

By support type



Amounts are Year 1 RHTP dollars parsed line by line from each applicant's own financial schedule. Lines whose staff type was written ambiguously were mapped only where the wording was unmistakable, such as 'Licensed Practicing Nurses' to LPN.

Awards across 28 parishes

RURAL CLINICIAN CREDIT BANK · Parish served · Home parish of each awardee

28

parishes with an awardee

48

awards with a single home parish

5

multi-parish awardees

Parishes by dollars awarded

Parish	Awards	Awarded	Parish	Awards	Awarded
Evangeline	4	\$1,217,199	Tangipahoa	2	\$458,612
St. Landry	6	\$1,200,414	Union	1	\$426,000
St. Martin	1	\$1,055,625	St. Helena	1	\$251,250
Caddo	1	\$775,190	Washington	2	\$217,500
Ascension	2	\$680,000	Acadia	4	\$211,023
Vermilion	2	\$679,500	Jackson	1	\$187,500
Pointe Coupee	3	\$643,125	Richland	2	\$175,000
LaSalle	1	\$623,750	Franklin	1	\$168,750
Beauregard	2	\$606,395	Concordia	1	\$118,125
Webster	1	\$472,500	Other 9 parishes	10	\$568,750

Parish is the awardee's own physical location as stated in its application. Most awardees serve several parishes beyond the one they sit in, so this maps where the money lands, not the full service footprint.



Multi-parish awardees

Awardee	Parishes	Award
Ochsner Clinic Foundation	8	\$1,500,000
Outpatient Medical Center	6	\$292,500
Winnsboro Medical Clinic	7	\$116,288
SR Minden, Southern Roots	4	\$45,000
LaBorde Therapy Center	4	\$12,000
Total		\$1,965,788

Concentration

Six parishes carry 44 % of the awarded dollars: Evangeline, St. Landry, St. Martin, Caddo, Ascension and Vermilion.

St. Landry has the most awards at 6. Acadia has 4 awards but only \$211,023.

The Acadiana corridor is awarded the most dollars.

Northeast Louisiana is thin: Union, Richland, Franklin and Jackson together take \$957,250.



RHTP: EHR Voucher initiative update

September 3rd, 2026



Today's agenda

- 1 Voucher program overview
- 2 Funding eligibility and timing
- 3 Participation expectations (CEA)
- 4 Fiscal year 2027 voucher timeline

1. Overview | RHTP EHR voucher program

PRELIMINARY – AS OF 9.1

RHTP will reimburse eligible EHR migration costs for rural providers adopting modern, Office of the National Coordinator for Health Information Technology (ONC)-certified EHRs, improving interoperability and supporting better care delivery

Key program characteristics:

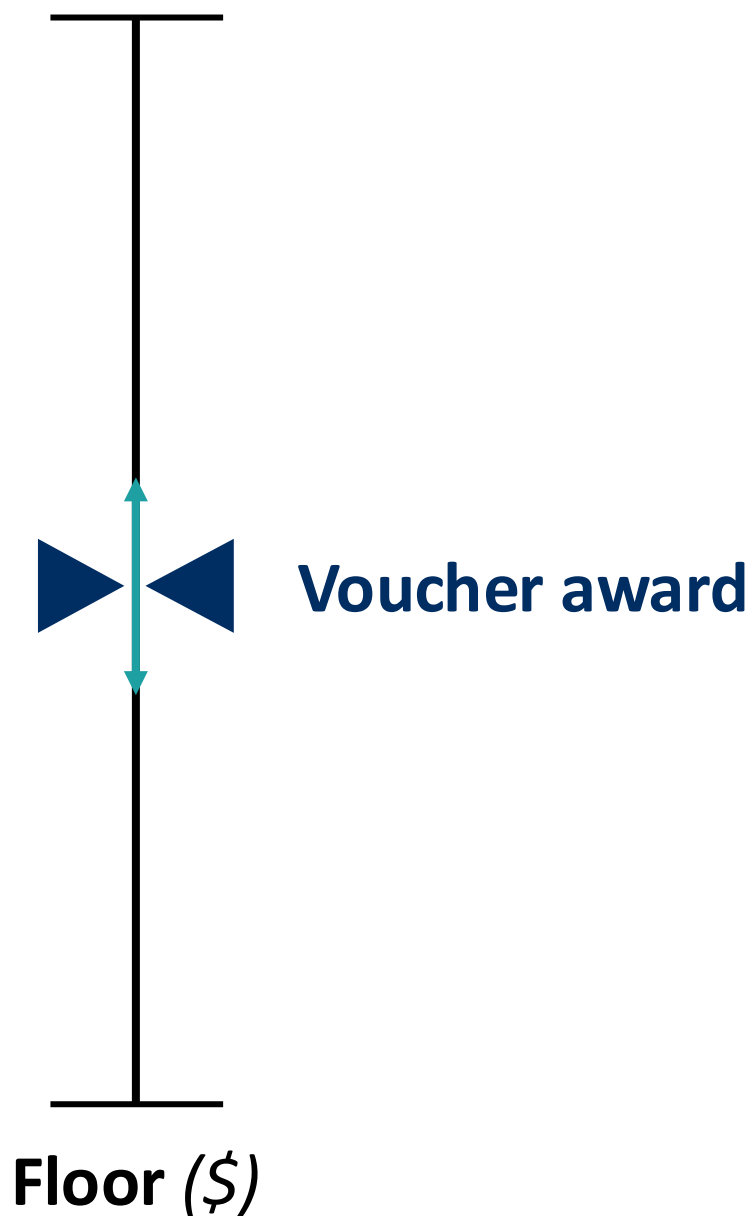
- **Reimbursement amounts will be determined using a transparent, auditable formula** that accounts for provider-specific characteristics and establishes a maximum award for each provider
- **Eligible migration expenses will be clearly defined**, with prescriptive guidance on which costs qualify for reimbursement
- **All rural providers will be eligible to apply**, with interest captured through a Notice of Funding Opportunity (NOFO)
- **Providers must migrate to an ONC-certified EHR**, consistent with CMS requirements for RHTP funding
- **Providers will have flexibility in their migration approach**, either adopting a self-managed EHR or joining a Louisiana-hosted EHR extension program (e.g., Community Connect)

2. The EHR voucher funding amount is driven by three characteristics of your facility

PRELIMINARY – AS OF 9.1

Maximum funding award per provider is assessed through a transparent, auditable formula:

Ceiling (75% of cost, \$\$\$)



Your voucher award is driven by three characteristics:

- 1. **Volume: Aligns funding to provider size,** closely correlating to EHR cost and complexity
- 2. **Key healthcare services provided:** Ensures support directed to providers offering **crucial rural healthcare services**¹
- 3. **Medicaid & rural patient proportion:** Matches funds with RHTP’s core objective of **supporting rural and underserved populations**

2. LA RHTP has provided clear guidance on migration expenses eligible for CC partner reimbursement

PRELIMINARY – AS OF 9.1

Legend:  Full voucher funding  Partial voucher funding (50%)  Supported via other initiatives  Not supported by RHTP funds

Migration cost category	Voucher funding eligibility
Software & licensing	 <i>Time-bound software & impl. costs reimbursed up to three years</i>
Professional services	 <i>Time-bound professional services supported up to 50%</i>
Internal labor	 <i>Time-bound internal labor supported up to 50%</i>
Hardware & equipment	 <i>Not eligible; potentially funded by Capital Improvement initiative</i>
Provider network & cybersecurity	 <i>Not eligible; potentially funded by Telehealth initiative</i>
Existing system maintenance	 <i>Maintenance of existing EHR during migration not eligible for RHTP reimbursement</i>

2. Example | EHR voucher support intended to reimburse portion of migration and M&O expenses

PRELIMINARY – AS OF 9.1

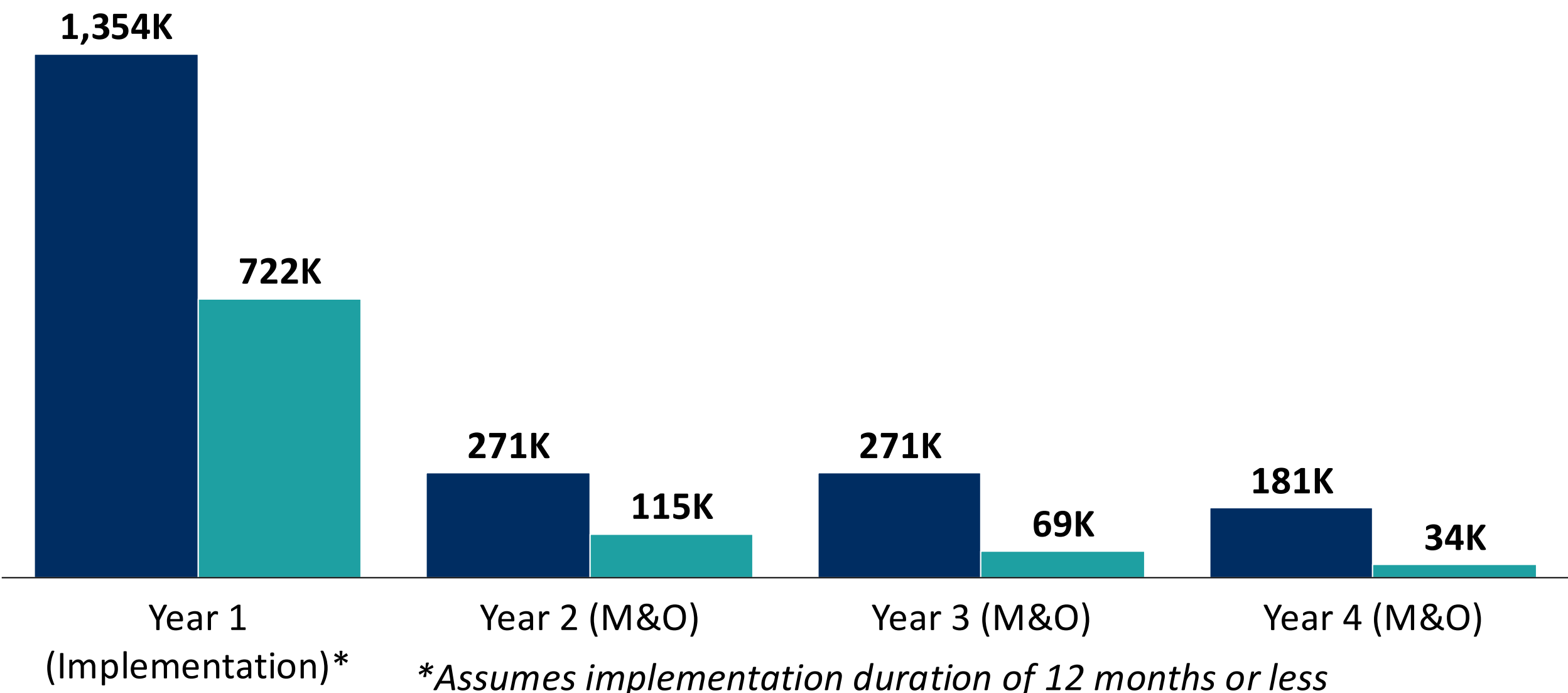
Example voucher recipient - stimated EHR migration costs and maximum voucher support

Illustrative Milestone:
Successful EHR go-live event

Illustrative Milestone:
75% of clinical workflows migrated

Illustrative Milestone:
90% of clinical workflows migrated

Illustrative Milestone:
100% of clinical workflows migrated



Est. migration cost

Voucher maximum

Key takeaways

- Funding eligibility occurs on an annual basis, requiring obligation prior to each reimbursement year
- Yearly milestones will be established to encourage provider adoption of EHR workflows
- Annual M&O voucher decreases over time, gradually transferring full cost to provider team after Year 3

3. A Cooperative Endeavor Agreement (CEA) will outline the expectations for participating rural providers

PRELIMINARY – AS OF 9.1

PROPOSED PARTNERSHIP – NOT LEGAL DOCUMENT

The CEA between LDH and providers formalizes provider participation in the RHTP EHR Voucher Program, establishing the voucher terms, participation commitments, and requirements to remain in good standing

Key CEA components

1. Participation expectations

- **Complete required onboarding, configuration, training, and implementation activities** and achieve on-time EHR go-live
- **Remain on the migrated EHR platform for at least three years** following go-live
- **The voucher is a one-time award;** each provider may receive only one LA RHTP EHR voucher

2. Reporting requirements

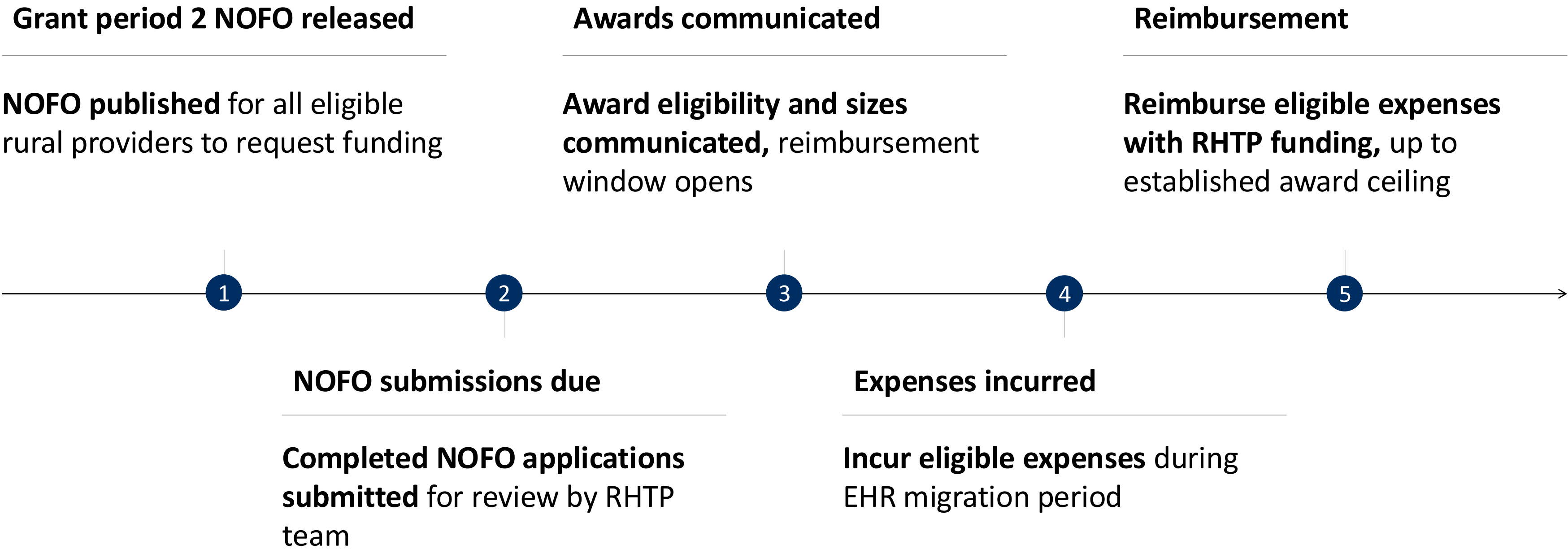
- **Demonstrate active EHR use through core workflow usage** (Clinical documentation, prescribing, etc.)
- **Support LDH reporting requirements,** including clinical quality measures, progress reporting, etc.
- **Maintain required interoperability capabilities** (e.g., HIE participation)

3. Deliverables

- Provide documentation to demonstrate go-live success, funding eligibility, and ongoing compliance, including:**
- Evidence of successful Go-Live
 - Reimbursement requests with required supporting documentation
 - Periodic implementation/progress reports and annual attestations

4. EHR voucher program sequence

PRELIMINARY – AS OF 9.1



Office of Rural Health Transformation and Sustainability

RuralHealthTransformation@la.gov

MAHARural@cms.hhs.gov